

**VA PORTLAND HEALTHCARE SYSTEM
PERFORMANCE WORK STATEMENT
CONTRACT MEDICAL RESPITE**

A. PURPOSE

The VA Portland Health Care System Department (VAPORHCS) requires Contractors to provide services as part of its community based Health Care for Homeless Veterans (HCHV) program. The HCHV Medical Respite program serves medically vulnerable Veterans experiencing homelessness who are in need of care above and beyond traditional transitional housing programs offered through HCHV and other services throughout the Portland metro area. The HCHV Medical Respite program supports Veterans discharging from the VA Portland Medical Center or other institutions with medical care needs, providing step-down care in an effort to reduce the need for continued hospitalization or use of emergency services.

B. BACKGROUND

The central goal of HCHV programs is to reduce homelessness among Veterans by conducting street outreach to Veterans who are homeless or experiencing housing instability and need case management assistance to end their homelessness and connect to community-based and VA-supported housing services, VA health care and other supportive services. In addition to outreach services, HCHV programs provide care, treatment, and rehabilitative services, including case management and therapeutic transitional housing assistance by contracting with community providers.

The Veteran must be enrolled and eligible for VA health care to receive HCHV contracted medical respite services. HCHV programs target and prioritize Veterans transitioning from literal street homelessness, homeless Veterans with co-occurring mental health or substance use disorder (SUDs), and Veterans who recently became homeless and require safe and stable living arrangements while seeking permanent housing or treatment options.

Federal law provides that VA medical facilities may establish contracts with community agencies to provide residential care to include therapeutic services and treatment to eligible Veterans. VA recognizes Contract Residential Service providers as independent and operating based on agreements as put forth in their contracts with VA medical facilities. VA provides guidance and oversight to ensure operations comply with Federal law and regulations and HCHV inspection standards.

Recent VA Portland HCHV medical respite contracts have served as a resource to homeless Veterans discharging from VA inpatient units and transitioning from literal homelessness, necessitating facility location to be within 15 miles of the VA Portland Medical Center, that can provide housing, medical stabilization services, and other psychosocial services to houseless Veterans. The emphasis is to continue serving the most vulnerable and at-risk homeless Veterans with a safe and secure environment, supporting their goals for medical recovery and providing access to housing programs for long term housing stability. The contractor will be required to provide a low-barrier therapeutic and rehabilitative milieu and attendant services to address medical rehabilitation. The contractor will not be required to provide detoxification or other acute level treatment. A Veteran using this service will be independent with all activities of daily living (ADL) and will not need personal assistance from contractor.

C. SERVICES TO BE PROVIDED

I. BASIC SERVICES:

- a. Secure and sanitary respite housing with available ADA-accessible dorm or single room as clinically indicated; limited to a maximum double occupancy unless approved by COR or designee.
- b. Access to onsite Primary Care and referral to outpatient Primary Care to support sustained medical treatment goals beyond Veterans stay in respite program. (Eg. Referral to VA PACT or initiating Medicaid services for community based provider depending on Veteran preference and eligibility.)
- c. Developing housing plan that may include referrals to transitional housing programs, recovery institutions, and services offering permanent supportive housing.
- d. Provision of basic necessities such as personal hygiene supplies, clothing and access to food including food boxes, meals-on-wheels, or other nutritionally appropriate resources.
- e. Case management for complex needs across health care, housing, treatment, employment and benefits systems. Support to make and keep appointments with Contractor and non-Contractor health care providers, including transportation.
- f. Development of a Veteran-centered discharge plan to support sustained wellness, recovery, treatment, and housing.

The contracted program provides services to all health care eligible Veterans who require recuperative medical care stabilization due to sickness, injury preparation, for and/or recovery from inpatient hospital care. It is understood that Veterans to be served under this contract will require care and services above the level of room and board.

The Contractor will provide therapeutic rehabilitative and recovery services, which shall be determined according to the needs of the Veteran through a biopsychosocial intake assessment. The Contractor shall be capable of receiving referral(s) Monday through Friday during business hours of 8am to 4:30pm, excluding Federal holidays with same day access when beds are available. Absences of any Veteran from the facility in excess of ninety-six (96) consecutive hours will not be reimbursable except with prior approval of the designated Contracting Officer's Representative (COR) or designee. Negative drug screens will not be a requirement for admission.

II. THERAPEUTIC AND REHABILITATIVE SERVICES:

a. Case Management:

- 1) Interventions will be Veteran-centered as they are tailored to the unique goals identified by each Veteran.
- 2) A comprehensive assessment of Veteran's psychosocial needs and Veteran-centered service plan will be documented within 7 business days of admission. Information from the screening and assessment can be integrated into the plan.
- 3) The service plan pertinent past treatment history, Veterans goals related to Veterans medical care, housing, and discharge plans.
- 4) Progress notes shall be reflective of progress toward treatment goals as they are updated throughout the Veteran's time in the program.

- 5) The Contractor will be expected to coordinate with the VA COR or designee to ensure Veterans goals, as expressed in the Contractor's intake assessment, are reflected in the ICP.
- 6) The Contractor is responsible for ensuring that requests for extension of services beyond the initially authorized service period of 90 days are reviewed and approved in writing by the VA COR or designee.
- 7) The Contractor will provide structured individual case management that includes supporting self-care skills around mental and physical health recovery, adaptive coping, financial planning, housing search, employment services, and referral for financial benefits.
- 8) The Contractor will document services and case management efforts by all team members including, but not limited to housing, medical treatment team, benefit and employment specialists, and program management staff. Progress updates will be communicated to the VA COR every 30-days.
- 9) Special attention will be made to address High Suicide Risk (HSR) Veterans as identified by the VA COR, VA Staff, and/or the Contractor. The Contractor will avail themselves to be contacted by the VA Suicide Prevention team immediately should a Veteran miss an appointment and his/her safety must be verified.

b. Medical Case Management Services:

- 1) Provides support to Veterans to maintain health and wellness.
- 2) Assists Veterans with scheduling medical and mental health appointments, provides customized education for medical diagnosis and medication management, and arranges transportation to and from all appointments. Makes referrals to community based organizations when health and/or psychosocial condition(s) indicate need for appropriate referral(s) and ensures all medications prescribed are reconciled to prevent duplication of orders.
- 3) Promotes clear communication amongst treating providers by ensuring awareness in regards to Veteran's medical needs and prescribed medication.
- 4) Provides coordination of care by meeting with Veterans and medical staff to determine goals and recovery targets for identified conditions.
- 5) Assist Veterans with medication management by monitoring compliance issues and providing medication education to improve compliance.
- 6) Teaches appropriate medical interventions in regard to wound care, self-care, and management of medical diagnosis.

c. Medication Management:

- 1) Contractor shall provide a secure and appropriate storage location for medication, including individual locked cabinet in shared room or access to a secure storage for medications that need to be stored at special temperatures, unless prior written approval received by VA COR.
- 2) Education and assistance with medication management shall be provided to Veteran by a licensed professional as needed.

d. Employment Services:

- 1) The Contractor will provide referrals to all relevant employment opportunities the Veteran is eligible for and interested in.
- 2) Employment readiness will be clearly assessed and documented.
- 3) The Contractor will document all structured activities that support Veteran in applying for employment.

e. Financial Planning:

- 1) The Contractor will refer to and follow up on all potential financial resources the Veteran may be eligible for (e.g. SSI, SSDI, SNAP, NSC pension, GA, etc.).

f. Recovery:

- 1) Support for a healthy lifestyle provided in an environment conducive to social interaction and the fullest development of the resident's rehabilitative potential.
- 2) Assistance with identifying resources and treatment options when clinically indicated and consistent with the Veteran's treatment goals. The Contractor is supportive of all recovery models.

g. Permanent Housing or Transitional Services Assistance:

- 1) The Contractor provides Veterans with direct and ongoing assistance in achieving permanent housing, transitional housing, recovery programs, and other housing placements including long term care when clinically indicated.
- 2) Details of Veterans progress toward housing goals shall be clearly reflected in progress notes, as well as, the care plan.
- 3) Examples of expected housing services can include:
 - i. Assistance obtaining and reviewing Veteran's free credit report, identifying associated housing strengths and/or barriers.
 - ii. Creating a Tenant Portfolio with Veteran that contains all relevant documents required in the rental process, including a completed sample rental application for Veteran, income verification documents, identification, applicable subsidies, etc.
 - iii. Reviewing affordable housing rental options and Permanent Supportive Housing (PSH) program offerings, and assisting Veterans with making appropriate rental inquiries and submitting completed rental applications.
 - iv. Conducting regular and individualized housing advocacy efforts, including transportation, assistance scheduling, meeting, and communicating with property managers and landlords, and troubleshoot rental barriers or denials occurring during the housing search.
 - v. Exploring affordable housing options .

h. Discharge Planning:

- 1) Securing housing stability plan will be the primary discharge goal for every Veteran.
- 2) All ICPs will be focused on short term goals that will include an initial plan for discharge. As with any placement, care plans change depending on resources available, clinical needs, and Veteran preference; this will be updated in documentation of services.

- 3) A negative discharge for undesirable behavior is a committee decision that ***must*** include a good faith effort to coordinate with the VA COR ***prior*** to discharge.
- 4) All Veterans scheduled for discharge based on behavior must meet with the treatment team and work with the team to determine most therapeutic option for Veteran.
 - a. *This does not include violations for safety reasons.* Any and all actual or threatened violence *will* be grounds for discharge. Veterans may be discharged for safety reasons at any time.
- 5) Performance expectation will be consistent with the VA National Scorecard (showing fiscal year 26 targets; subsequent changes and updates):
 - HCHV2 (DC w/ Neg. Exits) - 20%
 - Bed Occupancy > 80%
 - Length of Stay ≤ 90 Days
- i. **Staffing Ratio:**
 - 1) The Contractor will maintain a minimum staffing requirement ratio of one full time on-site case manager per 10 Veterans for special populations. Exceptions may be granted in writing by the VA COR.
 - 2) Staff is defined as a paid professional and does *not* include interns or trainees. The VA requires that case managers working with Veterans do not engage in any outside duties that do not pertain to the HCHV program which includes, case management of or group facilitation of non-Veteran populations unless a written waiver has been granted by the VA COR.
 - 3) The Contractor will have no more than two designated points of contact (POC) per program.

III. COORDINATION OF CARE

- a. **VA Coordination:**
 - 1) VA COR or designee is responsible for confirming program eligibility.
 - 2) VA COR or designee determines clinical appropriateness of placement and coordinates placement.
 - 3) Once approved for admission the VA COR will meet with Veteran within 7 days to complete necessary VA documentation.
 - 4) The Contractor completes all required VA endorsed paperwork
 - 5) The Contractor coordinates with VA COR as needed to support access to VA programs and services.
 - 6) The initial stay for a Veteran is expected to be no longer than 90 days.
 - 7) Program extensions are granted by the VA COR or designee and provided to the Contractor in writing every 30 days until a discharge is clinically appropriate.
 - 8) The Contractor will not be paid for care provided to a referred Veteran beyond the period authorized, unless an extension of the authorization is provided in writing by the VA
- b. **Intake Packets:**
 - 1) All Veterans must receive an intake packet within 72 business hours which includes, at minimum, the following information:

- i. Description of the Contractors services
- ii. Grievance Policy
- iii. Emergency procedures
- iv. Patient Rights and Reasonable Accommodation

c. Grievance Procedures:

- 1) The Contractor must have an internal grievance process that Veterans can use to resolve conflicts within the program. Policy must include written procedures for resolving grievances, including a statement regarding the client's right to request reasonable accommodation, and must post them in a conspicuous and accessible location. In addition, each client shall receive a copy of the grievance policies and procedures, upon intake and upon receiving a warning or discharge notice.

d. Incident Reporting:

- 1) The Contractor shall notify the VA immediately when *any* adverse critical incident involving a Veteran admitted to the program occurs. Critical Incidents include:
 - Falls
 - Fire
 - Medical or Psychiatric Emergency (9-1-1 Calls)
 - Hospitalization
 - Suicidal ideation or attempt
 - Assault
 - Death
 - Infectious control concerns (bed bugs, tb, etc.)
 - Possession of weapons
- 2) During normal business hours, the VA COR or designee shall be contacted immediately by phone to report such an incident. A written report to the VA COR should follow within 24 business hours. The report should list known precipitating factors. The Contractor shall maintain a copy of all critical incident reports in the involved Veteran's individual case record.

e. Documentation of services:

The residential treatment setting must comply with the following requirements:

- 1) Confidentiality of Certain Medical Records (38 U.S.C 7332),
- 2) Confidentiality of Alcohol and Drug Abuse Patients Records (42 CFR, Part II)
- 3) There will be no re-disclosure of patient information by the Contractor.
- 4) All records shall be made available to the VA COR or designee, including the VAPORHCS Executive Leadership team.
- 5) Electronic communication sent to the VA COR shall use encrypted software.
- 6) As required by local Continuum of Care, documentation and/or reporting the Homeless Management Information System (HMIS).
- 7) It is agreed that the VA may readily have access to all records concerning the Veteran's care in the program. Upon discharge or death of the Veteran, records on

all VA beneficiaries will be retained by the program for a period of at least three years following termination of care (ref. VHA Records Control Schedule 10-1).

f. Referrals, Admissions And Eligibility:

- 1) The Contractor is not permitted to require written applications from referral sources and/or Veteran as condition of admission to the program unless pre-approval has been obtained from the VA COR.
- 2) The Contractor shall only receive referrals from the VA COR or designee.
- 3) The Contractor shall utilize a transparent admission process that includes written eligibility criteria that is fair, objective, and compliant with applicable State and Federal laws.
- 4) The Contractor shall employ a risk reduction approach, taking into consideration the unique circumstances for each Veteran to determine whether the program can meet the individual's needs while maintaining the program's safety, security, and integrity.
- 5) The Contractor shall provide verbal or written notification to the VA COR, upon request, of reasons for non-acceptance.
- 6) The eligibility information must be made available to clients at intake and staff must provide answers to questions about the admission criteria and process.
- 7) Contractors must have formal appeal procedures through which clients may appeal unfavorable admission or eligibility decisions and a copy of the policy for appeals is to be given to the Veteran at screening or intake.
- 8) For denials, a waiver for admission and intake procedures can be requested by the VA COR.
- 9) The admission process is subject to review and adjustment may occur based upon mutual agreement between the Contractor and the VA COR, at any time, to ensure accessibility for Veterans.
- 10) The VA COR shall assist with eligibility determination of Veterans *prior* to acceptance by the Contractor for services. The Contractor is responsible for requesting verification of eligibility from the VA COR or designee.
- 11) It is understood that the Contractor shall not be paid for care provided to a referred Veteran beyond the period authorized, unless a written authorization is provided by VA COR.
- 12) To be eligible for placement in emergency contract beds, all Veterans must be homeless or at imminent risk of becoming homeless and be eligible and registered for VA services.
- 13) The Contractor provides housing and or services to special Veteran populations, including the following demographics: medically compromised, sex offenders, seriously mentally ill, and those struggling with drug and alcohol addiction, as well as, those with legal histories.
- 14) Exclusions for sex offenders may be made based on population status (i.e. families) or location of program.
- 15) Veterans cannot be denied entry by the Contractor based upon length of current abstinence from alcohol, active use, or history of substance use. In addition, Veterans shall not be denied entry due to their historical use of medical respite programs, disability, income, gender, or legal history.

IV. **FACILITIES:**

a. **Room and Board:**

- 1) Shall be accessible to the Veteran 7 days a week and 24 hours per day.
- 2) Accommodations will include a bed and other furnishings such as a dresser, storage locker and lock or designated locked secured space, and personal linens (towels, pillows, blankets and bed sheets, etc.).
- 3) Storage space will accommodate two bags of belongings and all prescribed medical equipment.
- 4) The Contractor will allow Veteran to store personal belongings for at least 72 hours after formal HCHV discharge.
- 5) Should a Veteran abandon his/her belongings, the contractor will make two documented attempts to contact the Veteran before disposing Veterans items. It is expected that Veterans will be briefed on this policy by the Contractor upon admission into the program.
- 6) The Contractor shall provide a sanitary food preparation area, supply food sufficient for at least three nutritious meals 7 days a week, and snacks of nourishing quality (e.g., fruits, vegetables, protein sources), between meals and bedtime.
- 7) As needed, the Contractor shall provide alternative meals or food supplies for Veterans with dietary restrictions if medically indicated (e.g., diabetic, renal, and soft mechanical diets) and reasonable accommodation for Veterans with cultural/religious preferences around food (e.g., Kosher, Sikh).
- 1) The facility shall provide storage space in an onsite refrigerator for Veterans to store and freely access personal food.
- 2) The facility shall maintain sanitary equipment to store, prepare and serve food in a sanitary manner, including providing appropriate cleaning supplies as needed.

b. **Laundry facilities:**

- 1) Detergent free of charge and adequate for residents to do their own laundry or to have laundry done at minimum one time per week.
- 2) Should a Veteran have difficulty doing his or her own laundry, the facility will provide guidance.
- 3) Should a Veteran refuse to wash his or her clothes, this will not be grounds for discharge but rather be addressed as a therapeutic opportunity.

c. **Milieu:**

- 1) The Contractor shall cultivate a milieu of trust and safety.
- 2) The Contractor shall have reasonable rules governing day-to-day life and activities in the facility. The purpose of these rules is to promote transition, safety, cleanliness, and dignity.
- 3) Veterans receiving care in the medical respite contract are not required to participate in chores, recreation activities, or other social activities, including recovery meetings or religious groups, unless identified as a treatment goal in the care plan.

- 4) The Contractor may offer opportunities for socialization and other therapeutic focused groups, however, Veteran's non-participation in these activities will not be grounds for discharge.

V. RULES, POLICIES AND PROCEDURES:

a. Procedures for Negative Discharge or Sanction:

- 1) The Contractor shall clarify and provide in writing upon program entry, the rules in which behaviors constitute gross misconduct and are grounds for immediate discharge from the program and which would prompt a written warning if violated and potential discharge if violated repeatedly.
- 2) The Contractor shall describe the formal appeal procedures through which Veterans may appeal program regulations, sanctions or discharges. This information must be provided to Veterans in writing upon intake and shall be clear and easily understandable by clients. This information will include at minimum:
 - i. Timely due process provisions that do not result in immediate discharge may include opportunity for a case conference or appeal within reason.
 - ii. Notice of, and access to, formal appeal procedures
 - iii. Notice of the conditions or process for re-admission to the program.
 - iv. Reasonable efforts to provide an appropriate referral to another facility or appropriate level of care as needed
 - v. This does not apply to Veterans who are discharged for danger to others

VI. PERSONNEL:

- a. The Contractor shall assign personnel that by education, training, and when required certification or licensure, to be qualified to provide the basic and supplemental services required by this PWS.
- b. The Contractor will employ sufficient personnel to carry out the policies, contract responsibilities, and the program for the facility. Services will be overseen by RN Care Manager or other qualified medical personnel to provide care and coordination of care to support the medical/medication needs of every enrolled Veteran.
- c. The Contractor shall have appropriate and commensurate coverage for staff using authorized and unauthorized leave (e.g. holidays, sick leave, family care, etc.).
- d. The Contractor shall identify an alternative staff to provide temporary coverage on site, covering the same schedule as the case manager who is on leave with the same credentials. The VA COR or designees is expected to be notified of cross coverage if the primary point of contract(s) is absent.
- e. There must be, at a minimum, one staff member available for emergencies 24 hours a day, 7 days a week.
- f. It is expected that an administrative/clinical designee is available for consultation for emergencies 24 hours a day, 7 days a week.
- g. The Contractor shall perform criminal background checks on all staff members that work with children.
- h. The Contractor shall agree not to hire Staff who are currently employed by the VA homeless program or who have ended participation in a VA homeless program within the past six months.
- i. The Contractor shall have a policy prohibiting staff from establishing sexual relationships with Veterans receiving care in this contract, in addition to, previously enrolled Veterans.

- j. The Contractor shall identify key personnel under this contract, and provide the VA COR a description of the services to be provided by each. The VA COR may request a summary for each person's relevant skills and experience.
- k. During the first ninety (90) calendar days of contract performance, the Contractor shall make NO substitutions of key personnel unless the substitution is necessitated by illness, death, or termination of employment.
- l. The VA reserves the right to refuse or revoke acceptance of key personnel if personal or professional conduct, or lack of required skills or experience, jeopardizes patient care or interferes with the regular and ordinary operation of the facility.
- m. Temporary substitutions of key personnel shall be permitted in accordance with the Contractor's contingency plan. The Contractor's contingency plan to be utilized if personnel leave Contractor's employment or are unable to continue performance in accordance with the terms and conditions of the resulting contract must be submitted as part of the Contractor's offer.

VII. ABSENCES AND CANCELLATIONS:

- a. The Contractor shall notify the VA of unauthorized absences from the facility within 48 hrs. of Veterans absence.
- b. When a Veteran is absent from the Contractor's facility in an unauthorized manner, payment for services for that Veteran shall be continued for a maximum period of 72 hours, provided there is a documented active outreach attempt on the part of the Contractor's staff to return the Veteran to the facility and there is a reasonable belief that the Veteran will return.
- c. VA reserves the right to remove any or all Veterans from the facility at any time, without additional cost, when it is determined to be in the best interest of the VA or the Veteran.
- d. The Contractor shall notify the VA COR immediately when a medical emergency occurs that requires hospitalization of an enrolled Veteran.

VIII. CONTRACTOR STAFF CONDUCT/COMPLAINTS HANDLING:

- a. The Contractor and its personnel shall be expected to treat referred Veterans with dignity and respect and abides by standards of conduct mirroring those prescribed by current federal personnel regulations.
- b. The Contractor shall comply with the VA Patient's Bill of Rights.
- c. The VA reserves the right to exclude Contractor staff members from providing services to Veterans under this contract based on breaches of conduct, including conduct that jeopardizes patient care or interferes with the regular and ordinary operation of the facility. Breaches of conduct include intoxication or debilitation resulting from drug use, theft, patient abuse, dereliction or negligence in performing directed tasks, or other conduct resulting in formal complaints by Veterans or other Staff members to designated Government representatives. The Contractor and VA COR shall deal with issues raised concerning Contractor personnel conduct. The VA Contracting Officer shall be the final arbiter on questions of acceptability and in validating complaints.

IX. BASIC RIGHTS AND PROTECTION AGAINST DISCRIMINATION:

- a. Contractors must protect the rights and dignity of the individual or family served in all phases of service delivery. At a minimum, providers must afford each Veteran the following rights and protections.
- Veterans shall be permitted to exercise these rights without fear of reprisal.
 - Veterans are entitled to enjoy a safe and healthful environment in the program.
 - Veterans are entitled to be treated in a manner that respects their dignity, privacy and individuality.
 - Veterans with disabilities are entitled to reasonable accommodations under fair housing laws when such accommodations are necessary because of their disability
 - Veterans are entitled to remain in the program and not be involuntarily removed without reasonable notice, good cause, and just procedures
 - All program clients are entitled to just and standardized procedures for determining eligibility, admissions, sanctions and discharges, and resolving grievances
 - Veterans are entitled to reasonable privacy and confidential treatment of personal, social, financial, medical, mental and behavioral health records, except as necessary to further treatment, information and referral services and in compliance with the resident's consent to release information
 - Veterans are entitled to the full exercise of their civil, constitutional, and legal rights
 - Veterans shall have on-going opportunities to voice opinions, to participate in program operation and programming, and to make suggestions regarding programming and rules.
 - Veterans rights must be protected against all forms of discrimination, including those based on race, religious creed, color, national origin, ancestry, language, disability (physical or mental health), medical condition, marital status, familial status, age, gender, sexual preference, source of income, or political affiliation
 - Veterans shall receive a written policy indicating that harassment of clients and staff on the basis of race, religious creed, color, national origin, ancestry, language, disability (physical or mental health), medical condition, marital status, familial status, age, gender, sexual preference, source of income, or political affiliation will not be condoned nor tolerated
 - Contractors shall develop a written policy for all Veterans to support safe, secure and dignified case management, as well as, within reason, accommodate special requirements needed for privacy.
 - All Veterans will have access to sleeping accommodations and bathroom facilities 24/7.
 - The Contractor must post these policies in a conspicuous place using appropriate languages.
 - All policies and procedures will be in writing and subject to review by the VA COR.

X. REASONABLE ACCOMMODATION:

- a. Contractor must respect and reasonably accommodate personal and cultural differences associated with race, religious creed, color, national origin, ancestry, language, disability (physical or mental health), medical condition, marital status, familial status, age, gender, sexual preference, source of income, or political affiliation.

- b. At no point shall program access be denied because of an Veteran's disability. Information learned about the Veteran's disability from the assessment will not be used as grounds for discharge or other punishment and shall be used to guide client to appropriate services. Moreover, a program may not apply different rules to individuals because of their disabilities, unless the different rules are a result of granting a reasonable accommodation request made by an individual with a disability.
- c. The Contractor shall not ask questions about a client's disability, including the nature and severity of the disability or the treatment, symptoms, and medications related to the disability. *A Veteran is not obligated to reveal that s/he has a disability prior to admission.*
- d. Veterans with disabilities are entitled to reasonable accommodations. If a Veteran requests a change in a program's policy or procedure as an accommodation of his/her disability, the program should grant the accommodation when the accommodation is both reasonable and necessary because of the individual's disability. Such an accommodation request must be considered during any stage in the provision of a program, including at intake, during services, and during discharge proceedings. When a reasonable accommodation request is made, the program supervisor may request verification of the individual's disability.
- e. The Contractor shall develop a written policy for their process for reasonable accommodations within 30 days of the award of this contract. This will include:
 - 1. All requests for reasonable accommodation,
 - 2. Details of all steps taken to accommodate a Veteran's request for reasonable accommodation,
 - 3. Any explanation for denial of services and/or accommodation, and
 - 4. Any appropriate referrals to Veterans who are denied accommodation. All explanations are to be written on formal letterhead from program management to the Veteran and Liaison at the time of denial for services or accommodation. Contractor will be aware that violation of the ADA, Section 504 of the Rehabilitation Act and Fair Housing Laws may result in stopping admissions and/or termination of contract for convenience.

XI. TRANSPORTATION:

The Contractor shall assist the Veterans with round trip transportation to scheduled meetings, medical appointments, and hospital visits within normal business hours. The Contractor will be expected to help the Veteran access public transportation, including providing information and instructions necessary to enable Veterans to utilize public transportation. If VA staff determines that adequate public transportation is not available or appropriate for a Veteran, the Contractor shall arrange for alternative transport by car, taxi, or other means.

XII. FACILITY:

It is the responsibility of the Contractor to properly maintain its facilities and the VA shall have no responsibility for paying or reimbursing the Contractor for such expenses.

- a. The contract facility must:

- i. Pass VA NFPA inspection within 60 days of the award of this contract if they have not been previously awarded a contract. Contract will be terminated for convenience if facility does not pass NFPA inspection within 60 days.
 - ii. Have a current occupancy permit issued by the local and state governments in the jurisdiction where the facility is located.
 - iii. Be in compliance with existing standards of the National Fire Protection Act (NFPA), State and local safety codes, and/or State health and sanitation codes.
 - iv. Where applicable, be licensed under State or local authority.
 - v. Where applicable, be accredited by the State.
 - vi. Be kept clean free of dirt, grime, mold, or other hazardous substances and damaged noticeably detract from the overall appearance.
 - vii. Be equipped with first aid equipment and an evacuation plan in case of emergency.
 - viii. Have windows and doors that can be opened and closed in accordance with manufacturer standards.
 - ix. Have an on-going plan to address bed bug infestation.
 - x. Have ADA accessible units to serve disabled Veterans. If there are no ADA accessible units, the Contractor is required to come up with a plan on how to accommodate ADA needs.
- b. This policy must be a part of your written response to this solicitation.
- c. The Contractor facility must meet fire safety requirements, as follows:
 - i. The building must meet the requirements of the applicable residential occupancy chapters of the current version of NFPA 101, National Fire Protection Association's Life Safety Code. Any equivalencies or variances must be approved by VAPORHCS Director
 - ii. All residents in the facility must be mentally and physically capable of leaving the building, unaided, in the event of an emergency
 - iii. Fire exit drills must be held at least quarterly. Residents must be instructed in evacuation procedures. A written fire plan for evacuation in the event of fire shall be developed and reviewed annually. The plan shall outline the duties, responsibilities and actions to be taken by the staff and residents in the event of a fire emergency. Plan shall be implemented during fire exit drills.
 - iv. A written policy regarding tobacco smoking in the facility shall be established and enforced.
 - v. NFPA 10 Portable fire extinguishers shall be installed at the facility and NFPA will be used as guidance in selection and location requirements of fire extinguishers.
 - vi. Requirements for fire protection equipment and systems shall be in accordance with NFPA 101. All fire protection systems and equipment, such as the fire alarm system, smoke detectors, and portable extinguishers, shall be inspected, tested and maintained in accordance with the applicable NFPA fire codes and the results documented.
 - vii. The annual inspection by a VA team required by paragraph 7 shall include a fire and safety inspection conducted at the facility unless a review of past Department of Veterans Affairs inspections or inspections made by the local authorities indicates that a fire and safety inspection would not be necessary, in

which case the fire and safety inspection may be waived by the VA.

XIII. INSPECTION OF FACILITY AND PROGRAM:

- a. Prior to the award of a contract, as well as annually during the contract term, a multidisciplinary VA team consisting of a Social Worker, Dietitian, a representative of the VA Police, and a Facilities Management Safety Officer, Nursing and/or other subject matter experts as determined necessary by the medical center director or VA COR shall conduct a survey of the Contractor's facilities to be used to provide Veterans food, shelter, and therapeutic services to assure the facility provides quality care in a safe environment. Inspections may also be carried out at such other times as deemed necessary by the Department of Veterans Affairs.
- b. Inspections may also be carried out at such other times as deemed necessary by the Department of Veterans Affairs:
 - i. Unannounced visits by the VA COR shall occur quarterly and will monitor the following:
 - 1) A visual safety and sanitation inspection of the facility including meal preparation areas, fire exits, sleeping areas and medication storage. Any safety or sanitation deficiency must be addressed immediately.
 - 2) Review of the contract provider's emergency and disaster plans to ensure they are up to date, and that staff are trained in the procedures outlined in the plans.
 - 3) An audit of Veteran's clinical records selected randomly to ensure documentation of case management services and that other services are being provided as required by the contract terms.
 - 4) Review of any Veteran complaints to ensure that these have been resolved in a fair, impartial and consistent manner.
 - 5) The report and any issues that were found along with the resolution must be kept in the COR Administrative File for that program.
 - ii. Unannounced nutrition inspection shall occur bi-annually.
 - iii. cursory environmental reviews shall occur when the VA COR or designee is on-site at the facility.
- c. The Contractor shall be advised of the findings of the inspection team. If deficiencies are noted during any inspection, the Contractor will be provided a reasonable time (90 days) in writing to take corrective action and to notify the VA COR that the corrections have been made. A contract will not be awarded until noted deficiencies have been resolved. Failure by the Contractor to take corrective action within a reasonable time will be reported to the VA Contracting Officer. If corrections are not made to the satisfaction of the VA, the Contracting Officer will consult with the appropriate officials so that suitable arrangements can be made to discontinue plans to award a contract, or to discharge or transfer Veterans, and to terminate the existing contract, as appropriate.
- d. The inspection of the Contractor facilities will include inspection for conformity to the current Life Safety Code and will also include the following:
 - i. General observation of residents to determine if they maintain an acceptable level of personal hygiene and grooming.
 - ii. Assessment of whether the facility meets applicable fire, safety and sanitation standards.

- iii. Determining whether the facility is in attractive surroundings conducive to social interaction and the fullest development of the resident's rehabilitative potential.
- iv. Observation of facility operations to see if appropriate organized activity programs are available during waking hours (including evenings) and degree to which a high level of activity is observed in the facility, such as individual professional counseling, physical activities, assistance with health and personal hygiene.
- v. Seeking evidence of facility-community interaction, demonstrated by the nature of scheduled activities or by information about resident flow out of the facility, e.g., community activities, volunteers, local consumer services, etc.
- vi. Observation of staff behavior and interaction with residents to determine if they convey an attitude of genuine concern and caring.
- vii. Inspecting the types of meals and other nutrition provided to residents to see if appetizing, nutritionally adequate meals are provided in a setting, which encourages social interaction and if nutritious snacks between meals and bedtime are available for those requiring or desiring additional food, when it is not medically contraindicated.
- viii. Making a spot check of Veterans' records to ensure accuracy with respect to Veterans' length of stay and services provided to the Veterans.
- e. All Department of Veterans Affairs reports of inspection of residential facilities furnishing treatment and rehabilitation services to eligible Veterans shall, to the extent possible, be made available to all government agencies charged with the responsibility of licensing or otherwise regulating or inspecting such institutions.

XIV. EMERGENCY PROCEDURES:

- a. The Contractor will train all staff on emergency procedures and have written protocols that are posted to guide staff response to crises including, but not limited to, physical injury, resident suicide attempts, overdoses, and domestic or other violence.
- b. Contractors will have *at least one designated paid staff on site at all times* who has had training and orientation on emergency procedures.
- c. Contractor will promptly and appropriately respond to the medical/psychiatric problems of clients and staff. There will be first aid equipment and supplies for medical emergencies available at all times. These supplies must be checked regularly to ensure they are up to date and their location in the facility must be clearly marked.
- d. Contractor will have all emergency contact numbers posted.
- e. Universal precaution practices are used by Contractor to prevent transmission of diseases and are implemented under the presumption that blood and body fluids from any source are to be considered potentially infectious. Supplies necessary for maintaining universal precautions, such as sharps containers, must be available.

XV. BILLING:

- a. Unless specifically excluded in this contract, the per diem rate established will include the services listed in this document and will also include all services or supplies normally provided other residents by the facility without extra charge. Payments made by the VA under this contract shall constitute the TOTAL cost of care and housing of

the homeless Veterans. Payment is authorized starting from the first day of admission and does not include the day of discharge.

- b. The Department of Veterans Affairs (VA) will pay per diem up to a maximum of 48 consecutive hours for the unscheduled absence or 96 hours for the scheduled absence of a Veteran under the following conditions:
- c. Scheduled Absences. To receive payment, the absence must:
 - i. Be pre-planned, consistent with and support the Veteran's individual service plan (e.g., family reunification, short term medical, substance use disorder (SUD) or psychiatric treatment).
 - ii. Have the reason documented in the individual Veteran's case file, treatment record, or service plan.
 - iii. Not result in the bed being filled by the Contractor.
 - iv. Not be for a break or vacation from treatment.
 - v. Not be used for extended educational or employment circumstances.
 - vi. Not be used to create more than 4 consecutive days of absence.
- d. Unscheduled Absences. To receive payment for an unscheduled absence:
 - i. The provider must have evidence of active outreach to locate and reengage the Veteran and document the steps taken in the Veterans individual case file, treatment record, or service plan.
 - ii. The provider may not fill the bed.
 - iii. The Veteran must be discharged from the HCHV CRS program if not located within 48 hours.
- e. Inpatient Hospitalization. HCHV medical recuperative care or geriatric care programs also serve a specific subset of Veterans with complex needs. The per diem payment may be made to a medical recuperative or geriatric care provider if a Veteran requires inpatient medical or psychiatric treatment for a period of up to 7 days. The provider may not fill the bed during this time and should maintain contact with the Veteran, VA Liaison, VA and VA inpatient staff if authorized.
- f. Ineligible Veteran. When a Veteran is admitted to a HCHV CRS program and found to be ineligible for HCHV CRS, VA will pay for a maximum of 4 days from the day of admission to allow the provider and HCHV CRS Liaison time to locate and arrange alternate placement.
- g. The Contractor will submit billing to the Payment Center by the 9th of every month. Failure to make timely invoice submissions will be cause for a request for a corrective action and if invoicing issues continue to arise it will be grounds for termination for convenience.
- h. The Contractor shall be expected to provide a method to verify a Veteran's daily attendance in the program. Confirmation is to be submitted to the VA COR upon request to ensure accuracy of billing. All absences shall be clearly documented, including reason for any excused absence (e.g. medical needs). Payments may be withheld if the

Contractor is unable to verify Veterans attendance in the program when requested by VA COR or designee.

XVI. QUARTERLY REPORTING:

- a. The Contractor shall provide the VA COR with a written report of program activities on a Quarterly basis in accordance with the QASP. The report shall contain, at minimum, the following information:
 - i. Total Number of Veterans Served
 - ii. Data from QASP compliance tool
 - iii. Program Completion Rates
 - iv. Outcomes (e.g. # or % of veterans discharged to *independent* housing, increased income/benefits, occupancy, etc.)
 - v. Other pertinent information, such as: quality improvement projects, changes in staffing or business practices, systems or resource concerns, etc.
- b. These reports shall be maintained by the VA COR in the contract administrative file.

XVII. CORRECTIVE ACTION PLAN:

- a. Should the Contractor become subject to a Corrective Action Plan (CAP) for failure to meet expectations identified in the contract, the Contractor will be provided, in writing, notification of the status by the VA COR or designee. Depending on the nature of the violation the Contractor will submit a corrective action plan to the VA COR within a timeframe *designated* by the COR and based on severity of violation (e.g. Safety issues). The program will be monitored for performance. Failure to perform duties herein may result in contract termination.

XVIII. ADDITIONAL CONTRACT REQUIREMENTS:

- a. All services shall be provided at no additional charge to the stated per diem rate, except for those services specifically stated below to be Supplemental Services.

XIX. DELIVERABLES:

Deliverables	Due Dates
D.1. QUALITY CONTROL BUSINESS PLAN	Due upon solicitation close date
D.2. STAFFING AND SERVICE PLAN	Due upon solicitation close date
D.3. DOCUMENTATION OF SUPPORTIVE SERVICES:	Completed Veteran case record due upon case close out; due to VA upon request only.
D.4. CARE PLAN	Due in Veteran case record by day 7; due to VA upon request

D.5. CRITICAL INCIDENT REPORTS	Due within 24 business hours of a critical adverse event involving a Veteran; in writing within 48 hours.
D.6. EXTENSION REQUESTS	Due in writing prior to the expiration of any authorized period of service
D.7. DISCHARGE REPORTS	Due within 48 business hours of Veteran discharge
D.9. DAILY CENSUS SIGN-IN OR CALL-IN SHEET	Verification of attendance when requested by the VA COR.
D.10. INVOICES	Due to VA COR by the 9th day of the month immediately following the billing period.